

Name: ايتاس محمود محمد منصور
National ID: 28003302102222



تقرير طبي

السيد

تحية طبية وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الدخول :
-- --:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد أنه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Supportive and palliative Treatment

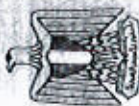
الوقت :

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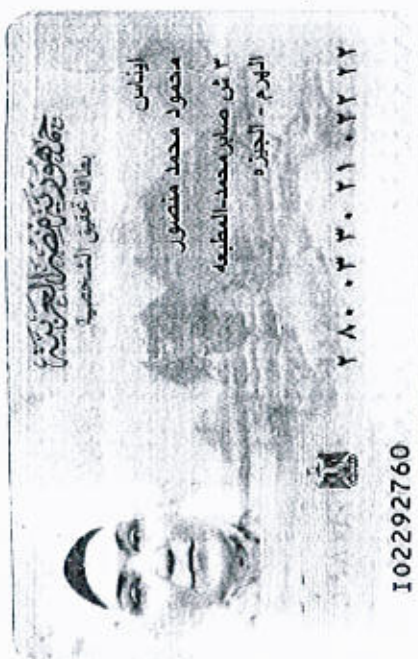
التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين



٢٠٢٠/٩/٢٠
٢٨٠٠٣٠٢١٠٢٢٢٢
حاصلة على بكالوريوس تجارة
متزوجة مسلمة
انثى
صالح محمود السقفي عبداللاه
البطاقة سارية حتى ٢٠٢٧/٠٩/١٦



HEALTH INSURANCE ORGANIZATION
CENTER DEPARTMENT OF
FORMATION AND DECISION SUPPORT



الهيئة العامة للتأمين الصحي
الإدارة المركزية لتكنولوجيا المعلومات
ودعم القرار

جهة الإصدار: فرع الج

تاريخ الإصدار: 2022-12-14

إلى من يهمه الأمر

الهيئة العامة للتأمين الصحي تعمل من خلال قوانين منظمة للانتفاع الخاضعين لقانون التأمين الاجتماعي رقم 79 لسنة 1975 المعدل بالقانون 25 لسنة 1977 "القوى العاملة والمعاشات والارامل" وكذلك قانون 99 لسنة 1992 الخاص بطلاب المدارس وقانون رقم 86 لسنة 2012 "الأطفال دون السن المدرسي" وقانون رقم 23 لسنة 2012 الخاص بالمرأة المعيلة وطبقاً لقاعدة البيانات المرسلة من التأمينات الاجتماعية وطبقاً لقاعدة البيانات المرسلة من الضمان الاجتماعي والبحث تبين أن

السيد (ة) ايناس محمود محمد بطاقة رقم 28003302102222

وطبقاً إلى خطاب من التأمينات الاجتماعية يفيد عدم الانتفاع
يعتبر من الفئات غير المنتفحة بالتأمين الصحي

وشكراً

تاريخ الطباعة: 2022-12-14

الموظف المختص

د. أمثال نعيم



إدارة مركز نظم المعلومات

تقدم،،،

مستند 256045



Name: <u>ايناس محمود محمد منصور</u>		
MR. : <u>3040</u>		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه		
The patient was initially staged clinically as T3 N1 with estrogen receptor–positive, progesterone receptor–positive, and HER2-positive breast cancer. A left modified radical mastectomy (MRM) was performed in 2020, followed by neoadjuvant chemotherapy and postoperative trastuzumab (Herceptin) administered subcutaneously. After completion of systemic therapy, the patient was placed on an aromatase inhibitor (Femara) and has now been on this endocrine therapy for five years. Ovarian ablation was performed in 2024.		
The patient continued on the same endocrine regimen (Femara) after ovarian ablation. A follow-up visit in November 2025 documented that the patient remained disease-free on physical examination (PAUS) and was presenting for routine Femara prescription renewal.		
A request was made for a chest X-ray and new laboratory studies, including alkaline phosphatase (ALP) monitoring while the patient remains on Femara for three additional months. Administration of zoledronic acid (Zometa) was planned for the following month.		
The patient received Femara today and was instructed to continue it for an additional three months. A computed tomography (CT) scan of the chest performed in March 2026 was reported as free of disease. Zoledronic acid (Zometa) was administered in the daycare pharmacy. Laboratory results showed serum creatinine of 0.66 mg/dL and ionized calcium (Ca++) of 1.10 mmol/L.		
The patient was seen for a bone scan on 13 June 2026, which was reported as free of disease. Femara was continued for another three months, and a thyroid ultrasound was performed as part of a new assessment. The patient remains on Femara with ongoing monitoring.		
تحريراً في ,		
	التاريخ :	20-09-2026
	الوقت :	
	الطبيب المعالج :	علياء احمد أبو النصر شهاب الدين
يعتمد ,		



Pathology Report

Name: ايناس محمود محمد

Age: 42 years.

Clinical data: Left breast cancer, post chemotherapy.

Specimen type: Left MRM.

Date: 29/8/2020

Lab. Number: 1241

Hospital Number: 291/2020

Gross Description:

MRM specimen, with skin covering area; 25x22x6 cm, including nipple & areola.

Serial cuts revealed diffuse rubbery greyish white area, admixed with fat.

Least resection margin (deep) was; 1.5 cm. There was no gross mass lesion or indurated foci (after careful sectioning)

Also received in the same container multiple mammary tissue pieces, the largest was;

4x3 cm, showed rubbery greyish white area, with focal yellowish foci measured: 2.5x2cm.

Together in the same container were received axillary tissue as multiple irregular pieces, collectively; 7x7 cm, entangling 16 nodules, up to; 1.2 cm in diameter.

Microscopic Examination:

Histopathological examination of sections prepared revealed breast tissue, with multicentric areas, showed mammary ducts, lined by moderately anaplastic ductal epithelial cells, with incomplete myoepithelial cells lining.

No invasive component.

No vascular tumor emboli.

No perineural invasion.

Free nipple areola & resection margins.

The axillary lymph nodes were free of metastatic deposits (0/16).

Diagnosis:

Left breast MRM, post chemotherapy;

- Multicentric residual DCIS, nuclear GH.
- No invasive component.
- No vascular tumor emboli.
- Free nipple, areola & resection margins.
- Free axillary lymph nodes (0/16).
- Complete pathological response to therapy.
- Stage, ypTis ypN0.

Prof. Dr. Amal Haridy
Dr. Enas Mohamed Ali

Prof. Dr. Essam Ayad
د. عصام عياد

ImmunohistoChemistry report

Report Number: m3849

Female :	النوع	إيناس محمود محمد	اسم المريض :
القاهرة :	العنوان	40 Y	السن :
21/07/2020 10:10 :	تاريخ استلام العينة		تاريخ اخذ العينة :
28/07/2020 11:05 :	تاريخ التقرير	مركز اورام دار السلام	طالب التحليل :
		مركز اورام نهار السلام	عناية :

Primary Sample :

Paraffin Block

Test :

ER & PR & HER2 neu & Ki67 (ER&PR: Harvey scoring system -Her2neu : CAP scoring system)

Comment :

Technique :

sample recieved was treated by: - CONFIRM anti-estrogen (ER) (SP1) Rabbit monoclonal primary antibody.- CONFIRM anti-progesterone receptor(PR) (1E2)Rabbit monoclonal primary antibody.

Sample recieved was treated by: -VENTANA anti-HER2/neu (4B5) Rabbit monoclonal primary antibody using DAB Detection kit in ventana benchmark GX.

Sample recieved was treated by: - CONFIRM anti Ki67 (30-9) Rabbeit monoclonal primary antibody.

Result :

ER: moderate nuclear staining of 60 % of tumour cells

PR: moderate nuclear staining of 10 % of tumour cells

HER2/neu protein over expression : No sufficient invasive tumour cells to comment on yet the intraduct tumour cells showed strong continuous membrane staining

Ki67 :positive nuclear staining of about 30-40 % of tumour cells

Referral Lab :

Comment :

Conclusion :

ER: POSITIVE (SCORE 6/8)

PR: POSITIVE (SCORE 4/8)

HER2/NEU PROTEIN OVER EXPRESSION : NO SUFFICIENT INVASIVE TUMOUR CELLS TO COMMENT ON (INTRADUC COMPONENT POSITIVE SCORE 3+)

#Ki67: THE PROLIFERATION INDEX OF THE TUMOUR IS ABOUT 30-40 %

(CORE BIOPSY)

Pathologist:

د/أيناس رمزي



Lab Director:

د/ تريزا ماهر



وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مركز أورام دار السلام
«هرمل» سابقاً

Name :

Age :

No.:

MDT FORM

Clinical data :

41 yrs old, A known
case of Lt breast Cancer.

received neoadj; con

AI x 4 => TAIL x 12

under

LT MRM + AI

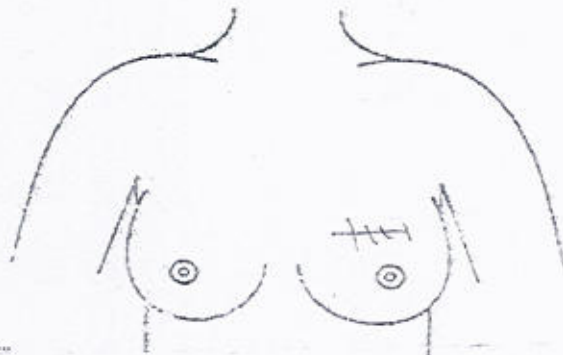
18/2/20

→ IOP, Cx

• ER

• PR +

• HER2 +



Reason of discussion: relined pretho + Tm

received Herceptin. x 8 Cycles

ended on 5/4/2021

MDT Outcome and Plan

Cont. of (Herceptin)

for 3 more
cycles

cont. to

Signature :

Signature of the patient

Signature of the physician

Signature of the pharmacist



POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

1. Chest:

- Bilateral enhancing breast parenchymal lesions are seen, largest at right lateral location measuring 2 x 2.2 cm and left medial location measuring 1.5 x 0.7 cm.
- No sizable pulmonary nodules, masses, areas of consolidation or cavitory lesions.
- subcentimetric prevascular LNs are seen.
- No pericardial effusion detected.
- Large hiatus hernia is seen.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is mildly enlarged with homogenous low CT texture. Small hypodense non-enhancing focal lesion is seen at segment VI measuring 7 x 5 mm.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Bulky uterus.
- No abdominal or pelvic lymph nodes or collections.
- The scanned skeletal portions are unremarkable.

OPINION:

- *Bilateral enhancing breast parenchymal lesions, for correlation with sono-mamographic findings.*
- *Fatty hepatomegaly with segment VI solitary small benign-featuring non-enhancing focal lesion.*

Dr. Peter Fahmy, MSc

Dr. Amr Farouk, MD

PATIENT NAME: ERAS MOHMOUD MOHAMMED
HANSOR
GENDER: Female
AGE: 39 y

PATIENT ID # 2019111649

ORDER # 656241

DATE OF SERVICE: 14-Jan-2020 01:51 PM

Report:

99MTC-MDP BONE SCAN

Desired Information: -
Skeletal survey.

Technique:

Delayed Whole body imaging in anterior and posterior projection performed 2 hours after I.V administration of 20 mCi 99m Tc-MDP in the following manner:

Report:

No hot spot that account for osseous deposits could be detected all over the surveyed skeleton.

Degenerative changes are seen over dorso-lumbar vertebrae.

Arthritic changes are seen over both knees.

Both kidneys are normally visualized.

Conclusion:

No definite evidence of metastatic osseous deposits.

Reported By,

Ass. Prof. Huda Fathy Elsayed

14-Jan-2020 04:20 PM